Pluvicto (Lutetium Lu 177 Vipivotide Tetraxetan)

Original Effective Date: 11/07/22
Revision Date(s): 11/07/22; 11/01/2023; 5/28/2024; 12/2/2024;4/17/2025
Review Date: 4/17/2025
Policy type: Medical Necessity
Line of Business: All GEHA Medical Health Plans

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan or the patients' unique circumstance, or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety and tolerability. Authorizations are for up to 6 doses per lifetime.

Indication Specific Approval Criteria:

Prostate Cancer, Metastatic

- Member is at least 18 years of age; **AND**
- Member's prostate cancer is castration resistant; **AND**
- Member has prostate-specific membrane antigen-positive disease; **AND**
- Used as a single agent[1]; **AND**
- Member has been treated with:
    - Androgen receptor pathway inhibitors[2]; **AND**
- Member has not received Radium-223 in prior 6 months; **AND**
- Member has not received prior PSMA-targeted radioligand therapy

Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to:

- National Comprehensive Cancer Network® (NCCN) - use consistent with NCCN recommendations carrying a Category 1 or 2A level of evidence; **OR**
- United States Food and Drug Administration (FDA) labeling - new drugs or regimens (combinations of drugs) consistent with all components of the product labeling; **OR**
- Indications not included in the official FDA labeling or recommended by NCCN (Category 1 or 2A level of evidence) may be considered if determined to be medically necessary per one or more of the following compendia:
    - Clinical Pharmacology (Strong For); **OR**
    - Wolters Kluwer Lexi-Drugs® (Level A); **OR**

[1] Members may continue androgen deprivation therapy (ADT) to maintain castrate levels of serum testosterone (<50 ng/dL).
[2] Examples include abiraterone, enzalutamide, darolutamide, or apalutamide. Abiraterone given as part of neoadjuvant/concomitant/adjuvant ADT with EBRT is not considered prior novel hormonal therapy.